

Lecture #97: OMM Lab 23 Prep Lecture: Counterstrain of the Shoulder, and Review Material (all Foundations Thoracic Region Material)

PPOM1 (Class of 2029)
March 23 to 27, 2026

To Shan Li, DO, Professor

Department of Osteopathic Manipulative Medicine

Do.
Make.
Heal.
Innovate.
Reinvent the Future.

Session Objectives

1. Students will review the principles of counterstrain and how to perform it.
2. Students will learn and practice treating the counterstrain points for supraspinatus, pectoralis minor, subscapularis, long head of biceps, short head of biceps/coracobrachialis, levator scapulae, and rhomboid minor/major.
3. Students will REVIEW the following: screening, diagnosis and MET Treatment of **AC**, **SC**, and **GH** joints (with MET additions to Spencer Technique), from PPOM-1 OMM Lab #22.
4. Students will REVIEW the following: thoracic diagnosis and ALL thoracic treatments from the Foundations course.
5. Review (ie. know) the autonomic innervation to the various organs that arise from the thoracic spine (see Foundations Week #1 lecture #2: OMM: Spinal Facilitation and Autonomic Nervous System; see Handout of ANS levels)

Counterstrain (CS):

- Counterstrain points are tenderpoints that can be treated with counterstrain.
- Passive indirect technique, making it extremely gentle and versatile.
- Originally called, “spontaneous release by positioning.”

Clinical pointers (worthwhile to remember these in practice):

- Due to its gentle nature, CS is very safe and versatile when properly performed for the right patient.
- Can be used in addition to other techniques to enhance results if needed.
- Patient’s symptoms may not be in the region of the CS point(s) but yet may be caused by them, so it is important to recognize when you should check for them.

Counterstrain treatment:

Indications:

1. Acute or chronic somatic dysfunctions
2. Somatic dysfunctions with a neural component such as a hyper-shortened muscle
3. As primary treatment or in conjunction with other approaches
4. Somatic dysfunctions in any area of the body (that may be related to the area being treated)

Contraindications (absolute):

1. Absence of somatic dysfunction
2. Lack of patient consent and/or cooperation.

Contraindications (relative):

1. Unable to voluntarily relax
2. Severe illness
3. Severe osteoporosis
4. Fortunately, contraindications are few. Always use good clinical judgement.

Counterstrain instructions for patients:

Advise your patients of the possibility (30-50% of the time) of a post-treatment “reaction”:

- An effective CS treatment *may* produce some pain (typically described as soreness) several hours after treatment; usually self limited (1-2 days) and well tolerated by patients – good idea to “warn them” so they expect it.
- Advise patient to avoid strenuous activity or strenuous exercise on the day of treatment (this is true for all OMT, not just CS)
- Increased water intake has been reported to help decrease likelihood of soreness to OMT.

Counterstrain treatment: Basic steps (reminders and tips)

1. Find the tender point with your monitoring finger. **Remember that the keys are that the point is tender, and to use your monitoring finger to detect soft tissue changes to guide you to the point (in practice, the location descriptions of the points are useful but are not the final determination of where the point is exactly located!)**
2. Set a relative pain scale for the patient by pressing on the point and calling it a 10/10. **This is not meant to rate the point more painful than it actually is. You are simply giving the patient an easy to remember starting point of 10/10.**
 - You can say something like, “We are going to call the pain you feel when I press on this point a 10 out of 10 in severity.”
3. Slowly place the patient passively in a position of ease that results in the greatest reduction of tenderness at the CS point (**“Fold and Hold” concept**).
 - a) You maintain the position of your finger on the point but ease up on your finger pressure while monitoring as you perform the counterstrain positioning to achieve this ideal position.
 - b) You press on the point to test the effectiveness of the treatment position and ask the patient “How does it feel now based the 10 we started with?” Remember to relax your finger pressure once the patient reports the tenderness.
 - c) FINE TUNE the position to achieve the greatest position of ease. **Aim to achieve a 0/10 rating (i.e. 100% reduction of tenderness). If it is not 0/10 then you should fine-tune further until at least a 3/10 rating is achieved. This may take one or more tries, but the idea is that you should make a reasonable number of attempts to achieve the maximal reduction in tenderness whenever possible. For example, if you fine-tune and get a 3/10 on the first try, why not try it again to see if you can reduce it further to a 2/10 or lower- make the 90 seconds you will spend in the treatment position count!**

Counterstrain treatment: Basic steps (reminders and tips)

4. Hold the final treatment position for 90 seconds while monitoring the point.
5. At the end of 90 seconds, slowly return the patient to the pre-treatment position *while maintaining contact* with the counterstrain point.
 - Prior to moving the patient, instruct the patient to “relax and do not help me”, then slowly return the patient to the initial neutral position. These steps will help avoid exciting the muscle spindles.
6. REASSESS the CS point for tenderness and tissue texture abnormality. **Press on the point and ask the patient, again, “How does it feel now based the 10 we started with?” You are looking for a 0/10. Achieving a 3/10 or less is considered a successful treatment of that point. If the tenderness is >3/10, then you may need or want to re-treat it with CS.**
 - ❖ Note that when we begin the technique, we **assign** the tenderness as 10/10 in order to have an easy to remember starting point. During the technique, as well as when reassessing after completing the technique, we are asking the patient to rate the tenderness on a 0-10 scale relative to the initially assigned level of 10. Again, consider retreating if tenderness is greater than 3 on the recheck.

Additional pointers for this lab:



1. Use proper set up such as appropriate table height, correct posture, and good control because these are all important to perform CS techniques comfortably and efficiently.
2. Gauge if your partner is truly relaxed. If the limb feels unusually light as you are holding them, it probably means that your partner is not completely relaxed and is holding the limb up for you.
3. Use the CS point scale- that is how you determine whether you have achieved the mobile point (i.e. ≤ 3 tenderness); remember to make a few attempts to get to 0/10 even though this is not always possible. Make sure your patient remains comfortable during this process (ie. avoid causing pain in CS point region or other region as you try to get to a 0/10; it's often not an issue but sometimes you have to compromise on how much you want to reduce the tenderness to). This is why you use small movements when fine-tuning.
4. Be mindful how far you move the limb when performing CS. Always tailor your treatment to your patient's unique tolerances.
5. Fine tuning is exactly what it sounds like- small incremental movements. Remember that combining different motions may allow you to achieve the mobile point more quickly and comfortably, without overusing the motion available in any one plane (which can cause discomfort if you move it too far).
6. As you perform CS, tune into your own body mechanics and get a sense of how it feels. You will realize that the described treatment positions are designed to allow you maximal control with the least amount of effort (if you doubt this, you can **as a learning exercise** change your hand or body position slightly and sense how it feels to you- you may discover that it doesn't feel as natural or may even be uncomfortable, in which case you should immediately stop and readjust your body and hand position).

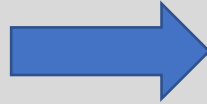
Always aim to:

- Correlate your *fine-tuning motions* with changes in the *tissue texture* felt with your monitoring finger
- Be aware of how much motion you are introducing into the joint(s) and avoid exceeding your partner's limitations
- Improve how you diagnose and treat through improved palpation and improved ergonomics. Learning new points is not just about having extra points you can use, it is about building your skills in delivering OMT (in this case, using CS).

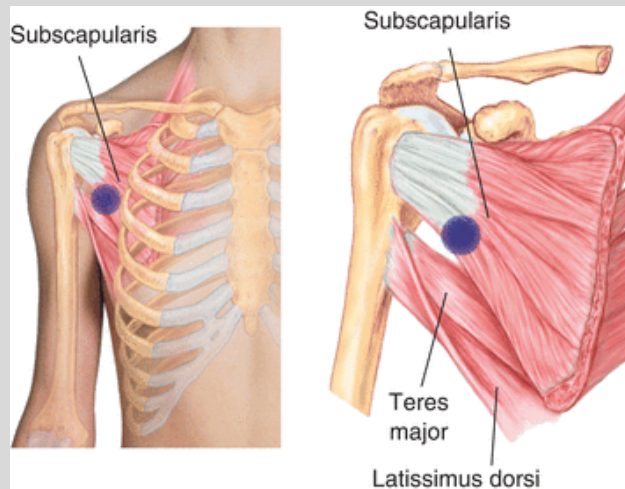
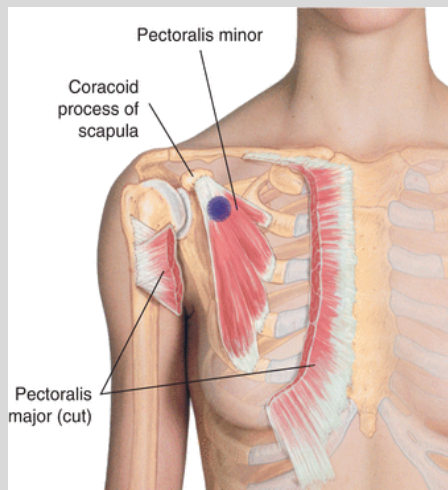
“That could be me some day!”

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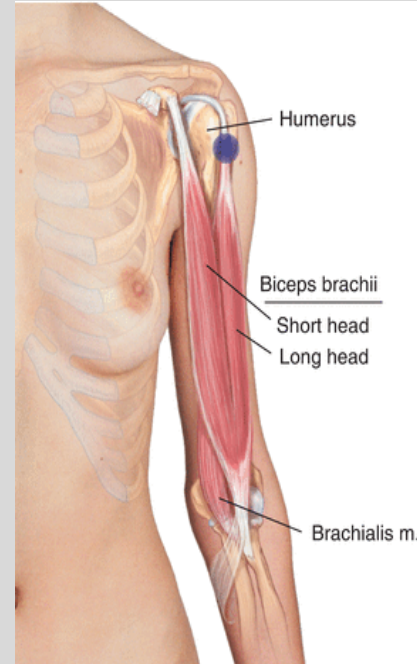
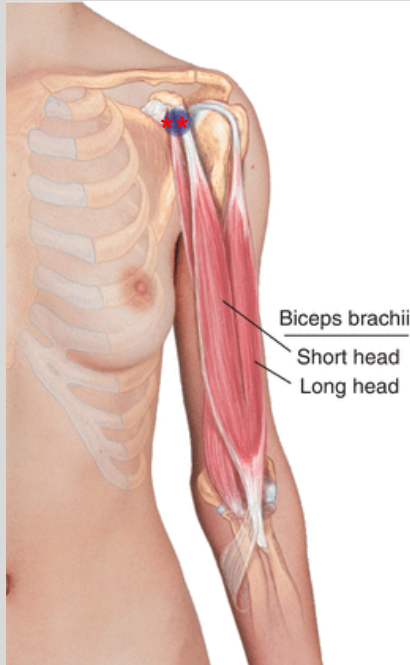
Shoulder Counterstrain points (Anteriorly located):



Nicholas A and Nicholas E. *Atlas of Osteopathic Techniques*, 3rd edition, Chp. 9- Counterstrain Techniques

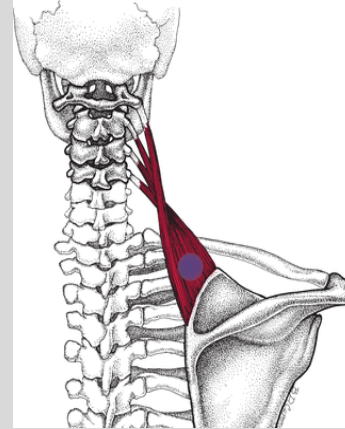
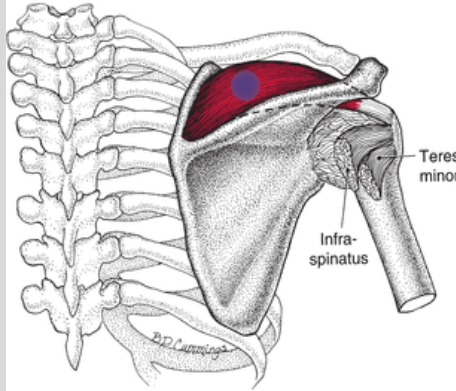
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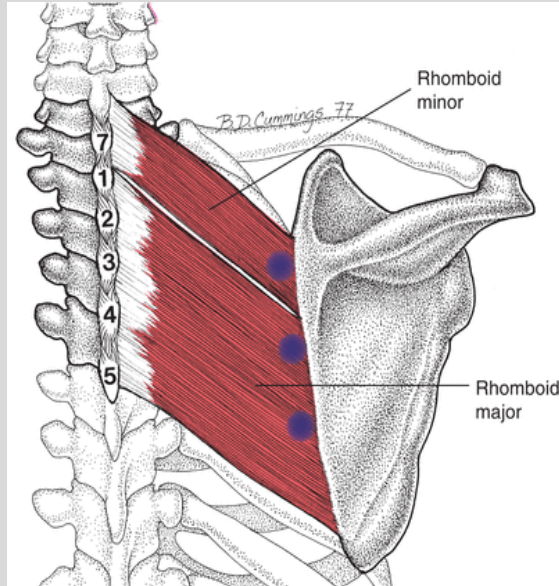


****May also be coracobrachialis CS point**

Shoulder Counterstrain points (Posteriorly located):



Shoulder Counterstrain points (Posteriorly located):



Final tips for Counterstrain

- Have a practice method for performing CS points:
 1. *Visualize* the final treatment position
 2. *Approach* CS logically. It is a “fold-and-hold” approach
 3. *Smoothly* set up and *sequence* through all the points
 4. *Take the time* to feel your body position and your monitoring finger as you find and maintain the mobile point

REVIEW:

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- **The above review material is testable on your written (thoracic) and practical exams (all of above) unless otherwise specified.**

Thank you

Lecture Feedback Form:

<https://comresearchdata.nyit.edu/redcap/surveys/?s=HRCY448FWYXREL4R>